

MEDICAL NURSING III)

1. The most distal part of the nephron is the:
 - A. Bowman's capsule
 - B. Collecting duct
 - C. Glomerulus
 - D. Loop of Henle
2. Which of the is not normally found in the glomerular filtrate?
 - A. Creatinine
 - B. Glucose
 - C. Protein
 - D. Uric acid
3. Which of the following is a muscle metabolism waste product is eliminated by the kidneys?
 - A. Creatine
 - B. Creatinine
 - C. Urea
 - D. Uric acid
4. Which electrolyte is reabsorbed in exchange for sodium?
 - A. Potassium
 - B. Chloride
 - C. calcium
 - D. Magnesium
5. Aldosterone contributes to blood pressure control by acting to increase
 - A. chloride elimination
 - B. potassium elimination
 - C. sodium elimination
 - D. sodium reabsorption
6. All the following are implied in nephrotic syndrome except:
 - A. Albumen
 - B. Globulin
 - C. Fluids
 - D. Glucose
7. The total number of orifices for outgoing and incoming of urine through the bladder is;
 - A. 1
 - B. 2
 - C. 3
 - D. 4
8. Mrs. Ankaah has just been rushed to the emergency unit of your facility with severe hemorrhage secondary to road traffic accident. She is at risk of developing acute renal failure which will be in nature.
 - A. intrarenal
 - B. perirenal
 - C. Postrenal
 - D. Prerenal
9. An abrupt decline in tubular and glomerular function due to exposure to nephrotoxins resulting in acute damage to renal tissue and nephrons will be classified as
 - A. Perirenal
 - B. Intrarenal
 - C. Postrenal
 - D. prerenal
10. Renal failure is caused by obstruction of urine flow will be classified as
 - A. intrarenal
 - B. perirenal
 - C. Postrenal
 - D. prevental
11. Which of the following is the treatment of choice for many clients with end-stage renal disease?
 - A. Continuous Renal Replacement Therapy
 - B. Hemodialysis
 - C. Kidney transplant
 - D. Peritoneal dialysis
12. In distinguishing acute from chronic renal failure, which of the following is the most reliable evidence of CKD?
 - A. Above-normal plasma sodium
 - B. Above-normal serum calcium
 - C. History of increased BUN
 - D. History of increased serum creatinine

13. Mr. Adika has been admitted to your unit with chronic renal failure. When completing the initial assessment which signs would indicate circulatory overload?
- An increase blood pressure, apprehension, and shock
 - Apprehension, poor tissue turgor, and bradycardia
 - Cool, dry skin, gastric distension, and pleural edema
 - Weight gain, coughing of frothy sputum, and jugular vein distention
14. Glomerulonephritis can occur as a complication of:
- Fungal infection
 - Poor diet
 - Straptococcal infection
 - Viral infection
15. Mr. Mensah who is on admission in your unit with nephrotic syndrome will indicate all the following clinical manifestations except:
- edema
 - .hematuria
 - hypertension
 - proteinuria
16. All the following are likely to be exhibited by patients with nephrotic syndrome except:
- Hypolipidemia
 - Proteinuria
 - Edema
 - Hypoalbuminemia
17. All the following dietary restrictions are relevant in the management of nephrotic syndrome except:
- Liberal fat intake
 - Low salt diet
 - Fluid restriction
 - Protein as kidney can tolerate
18. Which of the following conditions poses a danger for acid-base imbalance?
- Pancreatitis
 - Peptic ulcer disease
 - Pyloric stenosis:
 - Stomatitis
19. You have been assigned to prepare Mrs. Ed, who was diagnosed with pancreatitis, for discharge. Which of the following statements by the patient will suggest that she understands the discharge teaching?
- "I need to avoid eating for now"
 - "I need to minimize my fat intake"
 - "I need regular injections to digest food"
 - "My condition has no cure; I'll die anyway"
20. Which of the following nursing diagnosis is relevant when managing Master Addae with a diagnosis of pancreatitis?
- Altered body comfort
- Fluid volume excess
 - Body image disturbance
 - Fluid volume deficit
 - Altered body comfort
21. In the management of a patient with dental caries, patient should be educated to
- brush the teeth every morning.
 - eat their candies.
 - rinse the mouth each time after eating
 - snack as they wish.
22. Individuals with duodenal ulcers experience abdominal pain
- after eating.
 - at dawn.
 - in recumbent position
 - with fatty meals
23. Which of the following conditions may not affect the teeth?
- Cholecystitis
 - Gingivitis
 - Intestinal obstruction
 - Pyloric stenosis
24. Flunk pain is typical in which of the following conditions?
- Cholecystitis
 - Kidney infection
 - Hepatitis
 - Pancreatitis
25. Abdominal pain which is aggravated in the supine position and relieved by sitting up and leaning forward is most likely indicative of

- A. hepatitis b infection.
 - B. hepatitis c infection.
 - C. pancreatitis.
 - D. peptic ulcer disease.
26. Respiratory changes may be observed in a client with A ascites.
- A. Ascites .
 - B. Duodenal ulcers
 - C. gastric ulcen
 - D. stomatitis.
27. The transmission of which of the following will be curbed through safer sex techniques?
- A. Hepatitis A infection
 - B. Hepatitis B infection
 - C. Hepatitis E infotion
 - D. Pancreatitis
28. Which of the following conditions is likely to be characterized by changes in stool appearance?
- A. Liver disease.
 - B. Esophageal strictures.
 - C. Intestinal obstruction.
 - D. Pyloric stenosis.
29. Which of the following classes of medication should be eliminated from the drugs being taken by Miss Agidi who has just been diagnosed with gastric ulcers?
- A. Antacids
 - B. Hy receptor antagonists
 - C. ACE inhibitors
 - D. Proton-pump inhibitors
30. Which of the following is an He receptor antagonist?
- A. Cimetidine
 - B. Esomeprazole
 - C. Mist magnesium trisilicate
 - D. Omeprazole
31. All the following are possible reasons for nasogastric tube insertion in a patient with intestinal obstruction except to
- A. prevent aspiration
 - B. protect the esophagus
 - C. relieve patient of bloating
 - D. remove the obstruction
32. Black stool is indicative of
- A. anemia.
 - B. gastrie bleeding
 - C. lower GIT bleeding.
 - D. Rectal bleeding.
33. hit or the following types of lepatitis s mostly found in people with tepatis B
- A. Hepatitis A
 - B. Hepatitis B
 - C. Hepatitis C
 - D. Hepatitis D
34. When managing a patient with chronic hepatitis, which of the following is important?
- .. Interferon administration
 - 1. Paracetamol intake
 - 2. Administration of IV fluids
 - 3. Antibiotics administration
35. trol loving is not a complet of the mal reproductive system?
- A. Bartholin's glands
 - B. Bulbourethral glands
 - C. Prostate gland
 - D. Seminal vesicles
36. Mr. Ekutu, a 34-year-old taxi driver, is being managed at the OPD of your facility for hypertension. The history taken from him so far reveals that he has 3 wives. Additionally, he has admitted to having 2 girlfriends. He intentionally consumes a lot of salt, which you suspect is the reason for his increased blood pressure. Which of the following might be Mr. Ekutu's reason for consuming a lot of salt?
- A. Each of his wives add salt to his soup
 - B. He finds salty foods very tasty and attractive
 - C. He is not aware he eats a lot of salt
 - D. To boost the strength of his penile erections.
37. What is the most appropriate intervention to help Mr. Ekutu in the question above to reduce his salt intake?
- A. Advise him to divorce two of his wives and girlfriends
 - B Educate him on the dangers of excess salt intake

- C. Tell him to take his medications and he will be fine.
- D. Tell his wives and girlfriends to stop feeding with salt.
38. A newborn male child's scrotum must be examined for descended testes because
- A. only males with descended testes will find wives in later life.
- B. testes require 3°C lower temperature than the body to function efficiently.
- C. the midwife will be considered irresponsible if this is not done.
- D. there will be no space in the abdominopelvic cavity for the testes in adult life.
39. Which of the following interventions is not applicable in the management of acute prostatitis?
- A. Administration of analgesics
- B. Administration of antibiotics
- C. Administration of antipyretics
- D. Counselling the patient
40. According to the UPOINT system of prostatitis management, Nimplies
- A. administration of antibiotics.
- B. administration of neurogenic pain medications.
- C. non-steroidal anti-inflammatory drug administration.
- D. pelvic floor physical therapy.
41. All the following are possible clinical manifestations of orchitis except testicular
- A. chills.
- B. fever.
- C. malaise.
- D. redness.
42. After trying to conceive for a year, a couple consults an infertility specialist. When obtaining a history from the wife, Nurse Kwofie obtains past medical history. Which of the following diseases will most likely not affect the wife's fertility?
- A. Adrenal tumors
- B. Posterior pituitary tumors
- C. Sellar masses
- D. Testicular tumors
43. Mr. Adigbli has just received a prescription of ciprofloxacin 400 mg P.O. twice daily, for orchitis. The client asks the nurse how long to continue taking the drug. The nurse will ask the patient to take the medication
- A. as long as there are no side effects.
- B. for 7 days.
- C. until regimen is complete.
- D. when he feels sick.
44. Which of the following tests will help diagnose orchitis?
- A. DNA integrity test
- B. Post ejaculatory urinalysis
- C. Semen analysis
- D. Urethral swab
45. A client is embarrassed by urinary incontinence. Which of the following measures should Nurse Atipoe include in a urinary bladder retraining program?
- A. Assessing present elimination patterns.
- B. Encouraging the client to reduce time between voiding.
- C. Restricting fluid intake to reduce the need to void.
- D. Urethral catheter insertion to collect urine from the bladder
46. Which of the following is a male characteristic of erectile dysfunction?
- I. Being able to attain an erection but losing tumescence prior to penetration.
- II. Failure to achieve an erection from the outset of sexual activity.
- III. Losing tumescence during penetration.
- IV. Failure to satisfy sexual partner as desired.
- A. I & II only
- B. I, II, & III only
- C. I, II & IV only
- D. I, II, III & IV
47. Performance anxiety refers to fear of
- A. Criticism of the sexual partner
- B. Caring to appear as a failed erection.
- C. having sex in an unfamiliar place.
- D. not having an orgasm.
48. These cells of the testes secrete testosterone.
- A. Cells in the vas deferens
- B. Cells of germinal epithelium
- C. Cells of Leydig or interstitial cells
- D. Secondary spermatocytes
49. Another name for Cowper's gland isgland.
- A. Bulbourethral

- B.Meibomian
 - C.Perineal
 - D. Prostate
50. What is the name of the tube that carries sperm from the epididymis?
- A. Ejaculatory duct
 - B. Seminal vesicles
 - C. Urethra
 - D. Vas deferens

51. Sperm production takes place.... below core temperature.
- A. 13°C
 - B. 3 °C
 - C.5°C
 - D. 7°C

52. The size of the prostate gland normally
- A. decreases with increasing age.
 - B.increases with increasing age.
 - C.influenced by the brain.
 - D. remains same throughout life.

53. Enlargement of the gland can cause painful ejaculation.
- A. Bartholin's
 - B. Bulbourethral
 - C. Cowpers
 - D. Prostate

Section B: indicate whether the statements below are true or false by circling the letter corresponding to the correct answer. (17 Marks)

54. Occasional gastric acid reflux can predispose to development of GERD.
- A. True
 - B. False
55. Dental caries/s considered a nutritional-microbial disease.
- A. True
 - B. False
56. Vasopressin peptide influences kidney function.
- A. True
 - B. False
57. The most significant event in nephrotic syndrome is hematuria.
- A. True
 - B.False
58. Intake and output monitoring is a key component of patient management in pancreatitis.
- A. True
 - B. False
59. Females are to blame for majority of infertility cases.
- A. True
 - B. False
60. Calcium supplements should be considered in a patient on prolonged treatment with cimetidine.
- A. True
 - B.False
61. Vomitus in patients with pyloric stenosis can be stained with fecal matter.
- A. True
 - B False
62. Patients with renal conditions may also present with respiratory system symptoms.
- A.True
 - B. False
63. Pyloric stenosis is associated with immunocompromised state.
- A. True
 - B Falsu
64. Individuals with asthmatic attack have difficulty breathing in and out.
- A. True
 - B. False-
65. Kussmaul's respiration involves extended expiration and brief inspiration.
- A True
 - B. False
66. Liver failure can lead to anemia development.
- A. True
 - B. False
67. Asthmatic attacks may be triggered by exposure to pollen grains.
- A. True,

B. False

68. The term "semen" refers to a combination of spermatozoa and secretions from the male reproductive glands.

A. True

B. False

69. Patients with infertility may experience pain during ejaculation.

A. True

B. False

70. Pain related to gastroesophageal reflux disease is felt mostly in the hypogastric region.

A. True

B. False

SECTION C: indicate whether each of the options lettered A to E is true or not by writing T for true and F for false beside each option.

71. A patient with anasarca is given furosemide; the nurse must monitor the laboratory values to identify side effects of

- A. glucose.
- B. magnesium.
- C. potassium.
- D. sodium.
- E. sodium.

72. The pathophysiologic problem in liver cirrhosis causing esophageal varices is

- A. ascites and edema
- B. blood stasis in esophageal arteries
- C. dilated veins and varicosities
- D. loss of regeneration
- E. portal hypertension

73. Acute glomerulonephritis refers to a group of kidney diseases in which there is

- A. An antigen - antibody reaction to streptococcus
- B. An inflammatory reaction
- C. Cellular complex that lodge in the glomeruli and injure the kidney
- D. Sodium and water retention
- E. T-lymphocyte mediated kidney destruction

74. This is best used in the dietary management of edema.

- A. Food labels should be read carefully
- B. Low roughage diet
- C. Low sodium diet
- D. No added salt
- E. Use of seasoning substances

75. A patient is admitted into the hospital with a history of projectile vomiting after each feeding. Because of the patient's loss of gastric content, he will develop

- 1. acidosis. F.
- 2. alkalosis.
- 3. dehydration. 7
- 4. hypochloremia.
- 5. weakness. T

76. Sodium restriction is an effective therapeutic tool in the treatment of renal failure because it

- A. aids weak heart muscles to contract and increase cardiac output.
- B. allows excess fluid in the tissues to be excreted.
- C. eliminates the need for diuretic medication administration.
- D. helps to control food intake and body weight.
- E. helps to prevent potassium accumulation which occurs when sodium intake is high.

77. Nurse Adisa teaches a client with liver failure to take oral diuretics in the morning to help

- A. excrete excessive fluids accumulated at night
- B. improve diuretic medication efficiency
- C. prevention of electrolyte imbalance
- D. prevents sleep disturbances during night
- E. retard rapid drug absorption

78. A patient with peptic ulcer disease is put on antibacterial agents to

- A. augment the immune response.
- B. boost intrinsic factor secretion.
- C. potentiate the effect of antacids.
- D. reduce hydrochloric acid secretion.
- E. treat Helicobacter pylori infection.

SECTION D

Classify the etiologies below into PRERENAL, INTRARENAL and POSTRENAL causes of renal failure using the table below the list. (10 Marks) Shock, acute interstitial nephritis, Acute tubule necrosis, heart failure, prostatic hypertrophy, renal calculi, urethral stricture, angiotensin-converting enzyme inhibitors, increased uric acid production, radiographic contrast material.

PRERENAL	INTRARENAL	POSTRENAL

Below are the correct answers with clear nursing-focused rationales for MEDICAL–SURGICAL NURSING III.

I've structured them question by question so they are easy to revise for exams.

SECTION A: Multiple Choice Questions

1. Most distal part of the nephron

Answer: B. Collecting duct

Rationale: The collecting duct is the final segment where urine concentration is adjusted before excretion.

2. Not normally found in glomerular filtrate

Answer: C. Protein

Rationale: Proteins are too large to pass through the glomerular membrane.

3. Muscle metabolism waste eliminated by kidneys

Answer: B. Creatinine

Rationale: Creatinine is produced from muscle metabolism and is a key indicator of kidney function.

4. Electrolyte reabsorbed in exchange for sodium

Answer: A. Potassium

Rationale: Sodium reabsorption is coupled with potassium secretion, especially under aldosterone action.

5. Aldosterone increases

Answer: D. Sodium reabsorption

Rationale: Aldosterone increases sodium and water reabsorption, raising blood volume and pressure.

6. Not implied in nephrotic syndrome

Answer: D. Glucose

Rationale: Nephrotic syndrome involves protein loss, not glucose loss.

7. Total bladder orifices

Answer: C. 3

Rationale: Two ureteric openings + one urethral opening.

8. Renal failure from hemorrhage

Answer: D. Prerenal

Rationale: Hemorrhage causes reduced renal perfusion.

9. Nephrotoxin-induced renal damage

Answer: B. Intrarenal

Rationale: Nephrotoxins directly damage renal tissue.

10. Obstruction-related renal failure

Answer: C. Postrenal

Rationale: Urine flow obstruction causes postrenal failure.

11. Treatment of choice for ESRD

Answer: C. Kidney transplant

Rationale: Transplant offers the best long-term survival and quality of life.

12. Most reliable evidence of CKD

Answer: D. History of increased serum creatinine

Rationale: Persistent creatinine elevation indicates chronic damage.

13. Signs of circulatory overload

Answer: D. Weight gain, frothy sputum, JVD

Rationale: These indicate fluid overload and pulmonary edema.

14. Cause of glomerulonephritis

Answer: C. Streptococcal infection

Rationale: Post-streptococcal GN is common.

15. Not a feature of nephrotic syndrome

Answer: B. Hematuria

Rationale: Hematuria is more typical of nephritic syndrome.

16. Not seen in nephrotic syndrome

Answer: A. Hypolipidemia

Rationale: Nephrotic syndrome causes hyperlipidemia, not hypo.

17. Not a dietary restriction in nephrotic syndrome

Answer: A. Liberal fat intake

Rationale: Fat intake should be controlled due to hyperlipidemia.

18. Condition dangerous for acid-base imbalance

Answer: C. Pyloric stenosis

Rationale: Persistent vomiting causes metabolic alkalosis.

19. Correct discharge statement for pancreatitis

Answer: B. Minimize fat intake

Rationale: Fat stimulates pancreatic enzymes.

20. Priority nursing diagnosis in pancreatitis

Answer: 4. Altered body comfort

Rationale: Severe abdominal pain is the main complaint.

21. Dental caries education

Answer: C. Rinse mouth after eating

Rationale: Removes food particles and bacteria.

22. Duodenal ulcer pain

Answer: B. At dawn

Rationale: Pain occurs when stomach is empty.

23. Does not affect teeth

Answer: A. Cholecystitis

Rationale: Gallbladder disease has no dental impact.

24. Flank pain

Answer: B. Kidney infection

Rationale: Flank pain is classic for renal pathology.

25. Pain relieved by leaning forward

Answer: C. Pancreatitis

Rationale: This position reduces pancreatic pressure.

26. Respiratory changes seen in

Answer: A. Ascites

Rationale: Ascites restricts lung expansion.

27. Prevented by safer sex

Answer: B. Hepatitis B

Rationale: Transmitted sexually and via blood.

28. Stool appearance changes

Answer: A. Liver disease

Rationale: Causes pale or clay-colored stools.

29. Drug to eliminate in gastric ulcer

Answer: C. ACE inhibitors

Rationale: They do not aid ulcer healing.

30. H₂ receptor antagonist

Answer: A. Cimetidine

Rationale: Blocks histamine-2 receptors.

31. NG tube not used to

Answer: D. Remove obstruction

Rationale: NG tube decompresses, not removes blockage.

32. Black stool indicates

Answer: B. Gastric bleeding

Rationale: Melena results from upper GI bleeding.

33. Hepatitis associated with Hep B

Answer: D. Hepatitis D

Rationale: Hepatitis D requires Hepatitis B to replicate.

34. Important in chronic hepatitis

Answer: Interferon administration

Rationale: Reduces viral replication.

35. Not part of male reproductive system

Answer: A. Bartholin's glands

Rationale: Female reproductive glands.

36. Reason for excess salt intake

Answer: B. Likes salty food

Rationale: Taste preference is common.

37. Best intervention

Answer: B. Educate on dangers of excess salt

Rationale: Health education encourages behavior change.

38. Reason for descended testes

Answer: B. Require lower temperature

Rationale: Spermatogenesis needs cooler temperature.

39. Not used in acute prostatitis

Answer: D. Counselling

Rationale: Acute prostatitis requires medical therapy.

40. UPOINT – “N”

Answer: B. Neurogenic pain medications

41. Not a sign of orchitis

Answer: A. Chills

Rationale: Fever, redness, malaise are common.

42. Does not affect female fertility

Answer: D. Testicular tumors

43. Ciprofloxacin duration

Answer: C. Until regimen is complete

44. Diagnostic test for orchitis

Answer: D. Urethral swab

45. Bladder retraining

Answer: A. Assess elimination patterns

46. Erectile dysfunction characteristics

Answer: D. I, II, III & IV

47. Performance anxiety

Answer: B. Fear of losing erection

48. Testosterone-secreting cells

Answer: C. Leydig cells

49. Cowper's gland

Answer: A. Bulbourethral

50. Sperm-carrying tube

Answer: D. Vas deferens

51. Temperature difference for sperm

Answer: B. 3°C

52. Prostate size

Answer: B. Increases with age

53. Painful ejaculation

Answer: D. Prostate

SECTION B: TRUE / FALSE

True

True

True

False (Proteinuria is key)

True

False

True

False

True

False

True

False (Kussmaul = deep, rapid breathing)

True

True

True

True

False (Pain is epigastric)

SECTION C

71. Furosemide monitoring

A. F

B. T

C. T

D. T

E. T

72. Cause of esophageal varices

A. F

B. F

C. T

D. F

E. T

73. Acute glomerulonephritis

A. T

B. T

C. T

D. T

E. F

74. Dietary management of edema

A. T

B. F

C. T

D. T

E. F

75. Projectile vomiting causes

Acidosis – F

Alkalosis – T

Dehydration – T

Hypochloremia – T

Weakness – T

76. Sodium restriction

A. F

B. T

C. F

D. F

E. F

77. Morning diuretics

A. F

B. F

C. F

D. T

E. F

78. Antibiotics in PUD

A. F

B. F

C. F

D. F

E. T

SECTION D: Causes of Renal Failure

PRERENAL

INTRARENAL

POSTRENAL

Shock

Acute interstitial nephritis

Prostatic hypertrophy

Heart failure

Acute tubular necrosis

Renal calculi

ACE inhibitors

Increased uric acid

Urethral stricture

Contrast material